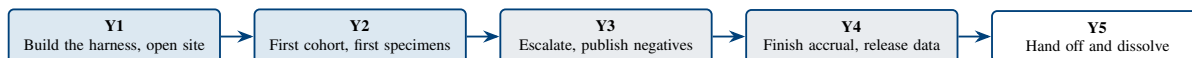

FRO Concept: A Time-Bound Organization for Perioperative RAS Evidence in Pancreatic Cancer



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Submitted as an independent scientist under the funding approach set out in Science: A New Golden Age (July 2026). Not medical or regulatory advice, and not endorsed by the FDA, NIH, HHS, an IRB, ICH, or any sponsor. UC San Diego is named as the intended partner of choice and is not a sponsor, site, or endorser.

1 Built to Dissolve

Chapter II of *Science: A New Golden Age* describes focused research organizations as time-bound nonprofit research startups built to dissolve, and places them in a gap it defines precisely: mid-scale science, tens of millions of dollars, coordinated teams of ten to a hundred, timelines of half a decade, which no existing institution fills [1]. The same chapter asks the roughly **\$200 billion** annual federal portfolio to stop deferring to the incumbents that consume it.

The output proposed here is a dataset, not a company and not a laboratory: what the RAS pathway does in human pancreatic tissue when a RAS(ON) multi-selective inhibitor is given around a curative-intent resection. It is too operationally heavy for a single academic laboratory, too unprofitable for a drug developer to fund on its own because it differentiates no product, and finished once it exists. That combination is the definition of an FRO deliverable shown in Table 1, and it is why this organization states its dissolution date in its first sentence.

FRO property	Instance here
A defined end state, not an institution	Organization closes in year 5 when the dataset and the harness are handed off
A deliverable no lab or company will produce	Specimen-level perioperative RAS pharmacodynamics, resected human PDAC
Mid-scale: small team, 1/2 decade	One scientist plus a contracted clinical site, five years, \$3.5 million
A named recipient for the output	Any investigator or sponsor designing a perioperative RAS study, has no licence

Table 1: Four defining properties of a focused research organization and the instance each takes here. The first row is a commitment, not an aspiration: the budget contains no year six.

2 The Gap the Dataset Fills

Daraxonrasib (RMC-6236) has a metastatic readout. RASolute 302 reported median overall survival of **13.2 months** against **6.6** for chemotherapy in the RAS G12 population of previously treated metastatic pancreatic cancer [2]. A ten-arm quantitative systems pharmacology simulation had returned **12.8** against **5.4 months** nine months earlier [3]; that proximity is chronology, not validation, and the three differences are sample size 1000 against 241, a combination against a single agent, and KRAS G12C against a primarily G12D and G12V population.

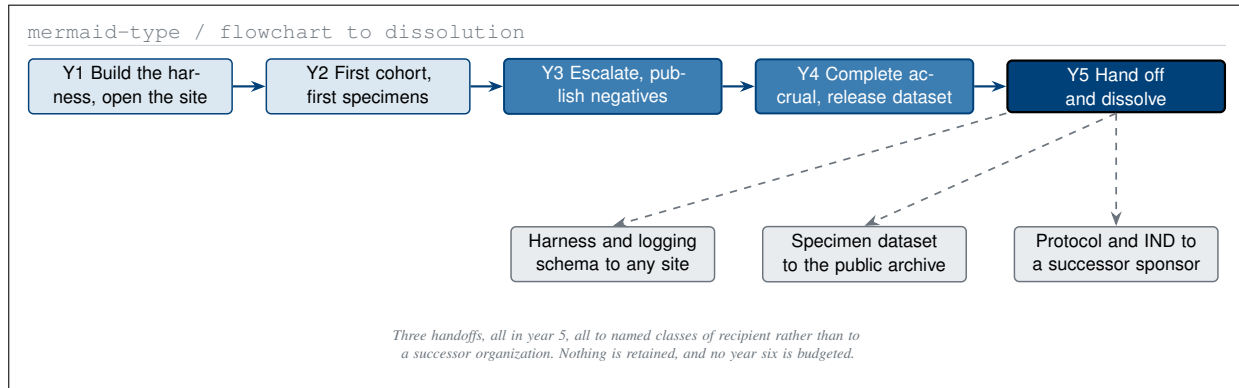


Figure 1: The five-year arc and its three year-five handoffs. The organization has no continuing operations after the handoffs, which is what distinguishes an FRO from a centre or an institute.

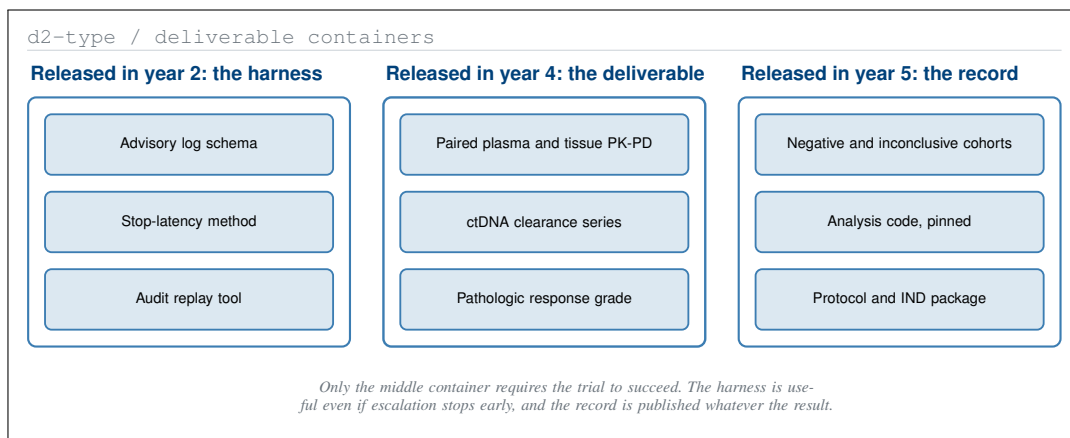


Figure 2: Three releases across the term, only one of which depends on the trial succeeding. An FRO that produces nothing unless its central hypothesis holds is not a well-designed FRO.

3 What Survives the Organization

The clinical vehicle is a Phase 1, open-label, single-arm 3+3 escalation of perioperative daraxonrasib in up to 18 participants with resectable or borderline-resectable KRAS G12-mutated PDAC, with a staged eight-arm robotic pancreaticoduodenectomy and an on-premises language model confined to advisory output [4, 5]. The organization dissolves; the following does not, as illustrated in Figure 1, 2.

Positioning note. First in human refers to the integrated surgical and advisory workflow, not to daraxonrasib, which is investigational and already in Phase 3 evaluation. The robotic configuration is specified by manufacturer, model, cleared configuration, arm count, and software version at the site agreement, and no drug supply or letter of authorization is in place with agent's developer.

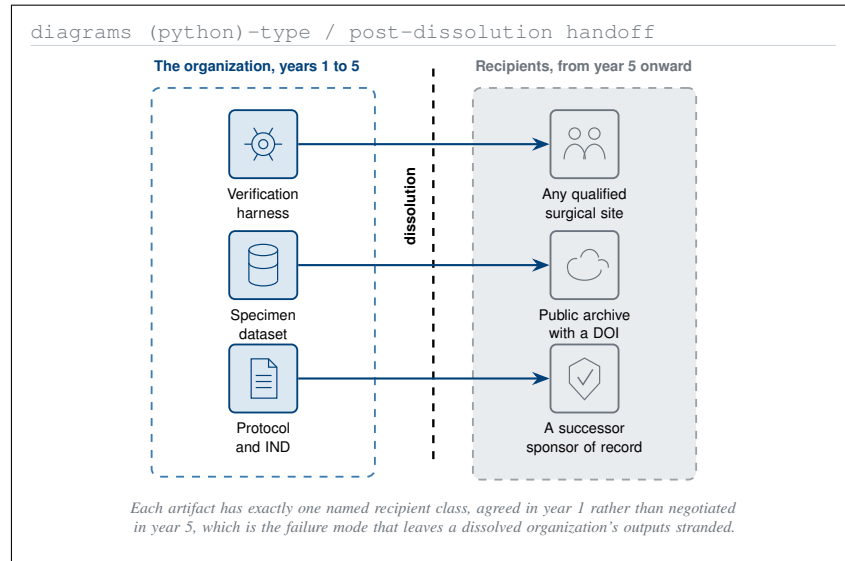


Figure 3: What crosses the dissolution boundary, and to whom. Recipient classes are named in year one, because negotiating them in year five is how a dissolved organization strands its own outputs.

4 Budget, Dissolution, and the Partner Site

The request in Table 2 is **\$700,000 per year for five years**, \$3,500,000 total, with no cost share and no sixth year [6], with additional details shown in Figure 3. The dissolution provision is budgeted rather than assumed: year five carries the archival, handoff, and closure costs explicitly.

Year	Direct cost	Principal activity	Release
1	\$700,000	Harness build, site agreement, IRB, IND activation	None
2	\$700,000	Cohort 1, first paired specimens, harness release	Harness
3	\$700,000	Escalation, interim audit, negative-result reporting	Interim negatives
4	\$700,000	Accrual complete, specimen dataset assembled	Dataset
5	\$700,000	Archival, three handoffs, closure and dissolution	Full record

Table 2: The five-year budget and its releases. Year five is a working year with its own deliverables rather than a wind-down, which is what keeps the dissolution from stranding the outputs.

Moore's Cancer Center is the intended partner of choice for the clinical work, approached at the feasibility stage only. UC San Diego is not a partner, sponsor, site, or endorser, because no written authorization exists.

Scope of Claims

Claimed: a deliverable no laboratory or company will produce on its own, three releases of which only one depends on the trial succeeding, and a dissolution provision that is budgeted rather than assumed. Not claimed: that the simulations validate RASolute 302, that the advisory system holds any regulatory clearance, that anything here has been used in a human, or that UC San Diego has agreed to anything. The trial does not exist until site, IRB, active IND, monitoring, insurance, and sponsor of record are in place.

Availability and Conflicts

Every work cited to a DOI is openly deposited; code, verification notebooks, and figure sources are in the repository [7]. The applicant holds no equity in Revolution Medicines or any robotic surgery vendor, and no sponsor, contract research organization, or medical society reviewed this document.

References

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